VALLEY GENERAL MEDICAL CENTER

Department of Internal Medicine | 1420 Harbor Blvd, Fullerton, CA 92831 | (714) 555-0180

PATIENT DISCHARGE INSTRUCTIONS

Patient:	NGUYEN, HUONG T.	**MRN:**	004-88-2317
DOB:	03/14/1955 (Age 71)	**Attending:**	R. Kaplan, MD
Admitted:	11/05/2026	**Discharged:**	11/08/2026 14:20

DISCHARGE DIAGNOSIS

Primary: Acute kidney injury (AKI), Stage 2, secondary to volume depletion, superimposed on Stage 3a chronic kidney disease. Secondary: Type 2 diabetes mellitus with diabetic nephropathy; essential hypertension.

MEDICATION INSTRUCTIONS

1. HOLD metformin 500 mg PO BID until you follow up with nephrology on 11/14. Do not resume unless instructed by the nephrology service.
2. HOLD lisinopril 20 mg PO daily pending repeat basic metabolic panel. Resumption is contingent upon creatinine normalization.
3. CONTINUE atorvastatin 40 mg PO QHS without interruption.
4. CONTINUE insulin glargine 18 units subcutaneously QHS. Monitor capillary blood glucose TID and prior to bedtime.
5. Avoid all NSAIDs, including ibuprofen and naproxen, as these agents are nephrotoxic and may precipitate recurrent renal injury.

ACTIVITY AND DIET

Ambulate as tolerated. Avoid strenuous exertion for 7 days. Adhere to a sodium-restricted diet (less than 2 g/day) and maintain adequate oral hydration, approximately 1.5 to 2 L daily unless otherwise directed. Restrict dietary potassium pending repeat electrolyte assessment.

FOLLOW-UP APPOINTMENTS

Nephrology, Dr. S. Almeida, 11/14/2026 at 9:30 AM, Suite 240. Obtain basic metabolic panel 48 hours prior to this appointment.

Primary Care, Dr. R. Kaplan, 11/21/2026 at 2:00 PM.

RETURN TO THE EMERGENCY DEPARTMENT IF YOU EXPERIENCE

Decreased urine output or anuria; new or worsening lower-extremity edema; dyspnea or orthopnea; persistent nausea and emesis precluding oral intake; confusion or altered mental status; or capillary blood glucose readings persistently below 70 mg/dL or above 300 mg/dL.

I have received and understand these instructions.

Patient / Representative signature _____ Date _____

Synthetic example document. Not a real patient record.